

February 20, 2026

Dr. Thomas Keane
Assistant Secretary for Technology Policy and
National Coordinator for Health Information Technology
U.S. Department of Health and Human Services
330 C Street SW
Washington, DC 20201

Re: RIN 0955-AA13 – Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care

Dear Dr. Keane,

On behalf of the HIMSS Electronic Health Record (EHR) Association, we value the opportunity to submit comments in response to the Request for Information (RFI) on actions the U.S. Department of Health and Human Services (HHS) can take to accelerate the adoption and use of artificial intelligence (AI) in clinical care.

Established in 2004, the EHR Association is comprised of nearly 30 member companies that supply the vast majority of EHRs used by hospitals, physician practices, and post-acute care environments across the United States. This gives us a comprehensive view of how AI-enabled tools are developing and being integrated into real-world clinical workflows, and this is the lens through which we respond to the RFI's questions below.

We remain committed to supporting safe healthcare delivery, fostering continued innovation, and operating with high integrity for EHR users and their patients. Our members are enthusiastic about the potential for AI in healthcare technology, with reasonable guardrails in place to protect patients and create a federal baseline for AI tools in certified health IT. We appreciate the chance this RFI presents to harmonize policies across HHS, avoiding conflicting obligations and reducing implementation burden. We also support efforts to modernize reimbursement models to reward clinically valuable AI that improves outcomes, reduces burden, and lowers the total cost of care.

We have developed the following core guiding principles for EHR Association member organizations to guide the design, development, and deployment of AI solutions; these inform our comments to the specific questions raised in the RFI:

AdvancedMD	Elation Health	Flatiron Health	MEDITECH, Inc.	Office Practicum
Altera Digital Health	Elekta	Foothold Technology	Modernizing Medicine	PointClickCare
Athenahealth	EndoSoft	Greenway Health	Netsmart	Sevocity
BestNotes	Epic	Harris Computer /MEDHOST	Nextech	TruBridge
CureMD	Experity	MatrixCare	NextGen Healthcare	Veradigm
eClinicalWorks				

- **Patient Safety and Clinical Efficacy:** We support the adoption of a national, risk-based governance framework that scales requirements (e.g., committees, processes) to organizational size and AI risk profile. AI must meet standards of safety, reliability, and clinical effectiveness commensurate with risk.
- **Clinician Oversight and Accountability:** AI should be designed and incorporated within EHRs and other health IT to augment informed clinical judgment, not be a substitute for a licensed clinical professional exercising judgment in a particular situation. Clinicians must be able to contextualize decisions or override outputs as appropriate. Regulations should affirm clinician primacy and permit context-driven overrides without mandating one-size-fits-all technical override functions for all AI modalities.
- **Transparency and Explainability:** Clear information about intended use, data provenance, performance characteristics, limitations, and monitoring responsibilities should be available for end users.
- **Privacy and Security:** Protection of identifiable health information compliant with not only HIPAA but also best practices for cybersecurity and data minimization.
- **Interoperability and Data Quality:** Adoption of standards-based data formats, provenance/traceability, and benchmarking to support safe, scalable deployment.
- **Human-Centered Design and Usability:** Workflow-aware design that reduces burden and supports safe use in context.
- **Lifecycle Risk Management:** Pre- and post-deployment evaluation, performance monitoring, and continuous improvement.

Responses to Specific RFI Questions

RFI Question 1 — Biggest barriers to private-sector AI innovation and adoption in clinical care

From the vantage point of EHR developers serving hospitals, ambulatory practices, and post-acute providers nationwide, the most significant barriers to AI innovation and adoption fall into five interconnected areas: regulatory fragmentation and uncertainty, data quality and access constraints, governance burdens that are not scaled to risk, misaligned payment incentives and unclear ROI, and workforce and workflow readiness.

First, providers and developers face ambiguity about how existing federal frameworks apply to non-device clinical AI versus FDA-regulated Software-as-a-Medical Device (SaMD), creating hesitation and elongated timelines for responsible deployment. A forward-leaning, harmonized HHS posture, coordinated across ASTP/ONC, FDA, CMS, and related offices, would materially reduce uncertainty, accelerate implementation, and promote trust, consistent with the RFI's objective of establishing a predictable, proportionate regulatory approach that both protects patients and enables innovation. The FDA guidance issued on January 6, 2026, on Clinical Decision Support Software is a helpful step toward greater certainty in this area, but we need to revisit it as AI technologies mature.

Second, even when use cases are clinically compelling, the foundation of data elements needed to safely build, validate, and monitor AI can sometimes be inconsistent, incomplete, or insufficiently representative of both diverse populations and care settings. The RFI's call to align federal levers so that AI is deployed in ways that enhance productivity, reduce burden, lower costs, and improve outcomes will depend on trustworthy, interoperable health data, ongoing oversight and retraining, and a regular cadence of standards enhancement and enforcement. To help remedy these issues, HHS could explore options for supplying rich, purpose-specific (and de-identified) data sets that the industry could rely on for high quality training and evaluation.

Third, many smaller organizations and rural providers face governance expectations in proposed legislation/frameworks that risk being overly burdensome. The EHR Association supports multi-disciplinary oversight but has cautioned that requiring rigid governance structures that do not reflect the specific needs of a given organization, costly external validation, and any too-specific timeline could impose disproportionate burdens on resource-constrained settings, as well as lower-risk AI functions that do not require the same scrutiny. We recommend establishing a tiered governance model that is scaled to the organization's type, the tool's intended use, and its risk profile, so that safety, fairness, and transparency are upheld without creating barriers that inadvertently discourage the adoption of beneficial technologies. A digital divide already exists specific to health IT between larger, more resource-rich healthcare systems and providers working in smaller ambulatory practices. We believe these smaller practices can measurably benefit from these technologies; supporting their adoption of these tools should be a priority.

Fourth, increased AI adoption would be best supported through aligned payment policies that reward clinical value, avoid burden imposition, and track total cost-of-care improvements. The RFI recognizes that legacy fee-for-service regimes frequently do not include mechanisms that incite adoption of innovative technologies, therefore slowing responsible uptake; modernized payment mechanisms that reward validated outcomes would catalyze provider demand and responsible scale.

Finally, workforce readiness remains a practical factor for consideration, as it does for any innovative but disruptive tool. For adoption of such a technology to be successful, clinicians and their staff need tools that smoothly fit within EHR-enabled workflows, and they also rely on both training offerings and organizational governance policies that allow them to confidently assume any associated risk. Provider organizations and our collective clients have made it clear that there is a critical need to preserve independent clinical judgment, including the ability to contextualize or override AI outputs. This aligns with positions put forward by the EHR Association, which state that AI should augment, not replace, clinician expertise. These conditions are essential to building trust, ensuring safety, and supporting continuous improvement at the point of care.

RFI Question 2: Priority Regulatory, Payment, and Programmatic Changes

From the perspective of the EHR Association, the most impactful actions HHS can take to accelerate responsible AI adoption involve establishing a clear, harmonized, and risk-based federal framework that allows developers and providers to innovate with confidence while ensuring that patient safety, clinical judgment, and system transparency remain foundational. The RFI underscores HHS's intent to create a regulatory posture that is predictable, proportionate to risk, and aligned with broader federal strategies to accelerate AI adoption in healthcare, reduce uncertainty, and promote trust.

In this context, we believe a formal federal framework specifically tailored to non-device clinical AI is urgently needed. Such a framework should articulate expectations for documentation, transparency, governance, and lifecycle monitoring while explicitly scaling requirements to the risk profile of the AI tool and the size and capacity of the implementing organization. Overly prescriptive governance structures can inadvertently burden smaller providers, and low-risk AI-supported tools, such as those focused on simplifying administrative tasks or reducing the burden on support staff, should be right-sized to support safe, equitable adoption. Given that many organizations have expressed a strong interest in adopting those low-risk models, any steps HHS can take within its regulatory framework to clearly differentiate between high- and low-risk AI implementations, with corresponding tiers of regulatory requirements, would be welcome.

A critical priority is aligning and harmonizing federal oversight across ASTP/ONC, FDA, CMS, and OCR, as well as attempting to harmonize the expanding patchwork of state laws regulating AI development and use. The HHS RFI itself acknowledges the need for a “forward-leaning, industry-supportive approach” that leverages regulation, reimbursement, and research to create clarity and accelerate adoption. Such an approach would go a long way toward alleviating the uncertainty that, in some cases, is creating hesitancy among both software developers and users.

The EHR Association strongly supports establishing shared definitions, common documentation artifacts, and coordinated expectations across agencies, particularly for intended-use statements, required performance information, and post-deployment monitoring responsibilities. Harmonization will significantly reduce duplicative compliance activities and help align clinical workflow integration, safety oversight, and privacy protections under a coherent national policy framework.

Payment modernization is also essential to ensuring that the value created by clinically validated AI tools is recognized and rewarded. The EHR Association encourages CMS to explore targeted add-on payments, AI-specific quality program credits, and pathways within alternative payment models (APMs) to support deployment of AI tools that improve clinical outcomes, reduce administrative burden, or enhance operational efficiency. Without clear, predictable financial signals, even highly effective tools may struggle to gain traction among financially constrained or operationally stretched providers.

Finally, the EHR Association recommends that HHS increase federal investment in benchmarking datasets, reference implementations, and safety testbeds to improve model transparency and external validation. Investments in shared safety testbeds and common reporting templates would further support trusted implementation and post-deployment monitoring across the industry.

Taken together, these regulatory, payment, and programmatic actions would significantly advance the responsible adoption of AI in clinical care while protecting patient safety, ensuring equitable application of data benefits to all patients, and rewarding clinician adoption of innovative technologies.

RFI Question 3: Novel Legal and Implementation Issues for Non-Medical-Device AI and HHS’s Role

From the EHR Association’s perspective, the rapid expansion of AI tools that influence clinical care (but do not fall under FDA regulation as medical devices) introduces a series of novel legal and operational challenges that require thoughtful federal guidance. A central issue is the allocation of liability among

developers, implementers, and clinicians when AI-generated recommendations inform care decisions. Because these tools usually operate in advisory or assistive roles and do not constitute regulated medical devices, the boundaries of responsibility can be ambiguous. To support safe deployment and reduce legal uncertainty, the industry would benefit from model “duty of care” frameworks and safe-harbor practices for documenting AI-informed clinical decisions, recognizing that the ultimate responsibility for patient care ultimately rests with the authorizing clinician. Such structures would help ensure that accountability aligns with real-world roles and capabilities while preserving clinicians’ independent judgment.

Another challenge arises in the contracting and indemnification frameworks needed for adaptive or continuously learning AI systems. Traditional contracting models do not adequately address issues such as performance warranties, update obligations, or the allocation of risks associated with model drift and re-training. There is work necessary for the industry to collectively identify best practices that lay out reasonable expectations for developers, implementers, and users, including shared responsibilities for updates, logic and data transparency, ongoing training and performance monitoring.

Non-device AI systems also pose novel privacy and security complexities and challenges for implementers to consider. While the applicability of and obligations under HIPAA Privacy and Security Rules are clear and consistent with traditional software, the strategies implementers should deploy to ensure compliance with those obligations are not always so clear. The industry as a whole could benefit greatly from thorough guidance on best practices for privacy and security considerations and strategies coming out of HHS in collaboration with federal partners, such as NIST.

Finally, the rise of AI-augmented clinical workflows raises workforce, credentialing, and scope-of-practice considerations. As clinicians integrate AI outputs into care decisions, organizations need to implement policies that clarify training, competency, and oversight expectations (and should make those expectations clear to their software partners as well). These issues also intersect with CMS Conditions of Participation, clinical quality measurement programs, and HRSA workforce development initiatives, where HHS can play a direct role in ensuring both resources are sufficient and expectations are clear. We also recommend that HHS consider investments in workforce training programs specific to AI safety and efficiency.

Taken together, these legal and operational complexities underscore the importance of coordinated federal leadership. By offering clear, consistent guidance on liability, contracting, privacy, and workforce readiness, HHS can significantly reduce uncertainty, support safe and equitable adoption, and build the foundation for responsible use of non-device AI tools across the healthcare ecosystem.

RFI Question 4: Promising evaluation methods for non-medical device AI and how HHS can support them

Non-medical device AI tools benefit from evaluation methods that combine traditional validation with real-world clinical usability and workflow safety assessments. While the industry is nascent with best practices still evolving, it is logical that structured intended-use analysis, external validation across representative patient populations, and human-factors testing in live or simulated EHR workflows are all steps to consider as stakeholders determine their optimal processes. Post-deployment, ongoing

monitoring for calibration, drift, fairness impacts, alert fatigue, and patient outcomes are all beneficial in ensuring a safe deployment of health-related AI.

Because these evaluation practices vary in maturity across the industry, HHS could add value by funding the development of common benchmarks, publishing reporting templates for industry consideration, and supporting shared voluntary testbeds that would enable transparent, replicable assessment across organizations and care settings.

RFI Question 5: Supporting private-sector accreditation, certification, and testing

Because there is no federal framework for AI development and use currently in place, many private-sector EHR developers have already taken substantial steps to establish internal governance, testing, and safety protocols. In some cases, our customers have communicated that they also require safeguards and data testing before proceeding with adopting AI tools and functionality.

We believe the private sector will play an active role in the conversation specific to ideas about voluntary accreditation, certification, and testing mechanisms for clinical AI. However, standardizing expectations for model audit trails will help ensure that AI-supported decisions are consistent in key ways across the industry, whether applied by EHR developers or independent AI model developers without certified EHRs on the market. Such consistency would likely lead to increased confidence in AI solutions and a more consistent framework for comparing different AI deployments across the healthcare industry for key metrics of safety, accuracy, and efficiency.

RFI Question 6: Where AI has met or fallen short, and high-impact opportunities

We believe it is possible to accelerate provider adoption of AI innovations if the capabilities are paired with payment models that reward validated clinical benefit.

RFI Question 7: Roles influencing adoption and primary administrative hurdles

Decisions about whether and how to adopt AI tools are influenced by a complex array of stakeholders within healthcare organizations, including clinical leadership, IT and EHR teams, quality and safety officers, compliance, and privacy teams, and financial or operational executives. Conversely, in smaller organizations, the decision-making authority may be vested in a single individual or a small cohort of clinicians. Despite interest, adoption is sometimes slowed by administrative hurdles, such as lengthy procurement processes, security reviews, and uncertainty about what documentation would be required of governance structures must be established.

RFI Question 8: Interoperability enhancements to widen opportunities

The EHR Association emphasizes that progress in AI adoption is directly linked to the quality and consistency of interoperability across the health IT ecosystem. AI tools depend on accessibility to structured, standards-based data to generate reliable and equitable predictions, but that must be coupled with data provenance transparency and audit trails that capture data use and exchange for purposes of ensuring sufficient monitoring capability. This would contribute to the expansion of opportunities for innovation and improve the reliability of external validation efforts. Investments in

national benchmarking datasets and standardized evaluation frameworks would further aid developers, researchers, and providers in assessing model performance across diverse populations and environments.

RFI Question 9: Patient and caregiver priorities and concerns

Patients and caregivers increasingly expect AI to be part of their care process, but they also sometimes raise concerns about privacy, bias, and accountability when AI-related errors occur. Additionally, some inquire about opting out of AI use in their care, and all are questions that healthcare providers must be prepared to address to maintain public trust. Providers should provide clear, accessible communication materials that help patients understand when and how AI is used in their care and what options they have to ask questions, seek clarification, or raise concerns. HHS could also help by providing guidance on plain-language communications about AI use for providers to help inform patients about AI use, support opt-in/opt-out choices where feasible, and provide clear pathways for questions, appeals, and remediation.

RFI Question 10: Priority areas for AI research and the literature on costs/benefits

The EHR Association encourages HHS to prioritize research efforts that address the elements most critical to safe and equitable AI adoption: data and logic transparency, human-centered usability, real-world safety evaluation, and performance monitoring. We also support investment in pragmatic trial designs and implementation science approaches that measure AI's true impact on outcomes, clinician workload, equitable application of the technology, and total cost of care. Because cost-benefit analyses and burden-reduction measures are still emerging fields in AI evaluation, federally supported research in this area could help inform payment reform specific to AI use.

Conclusion

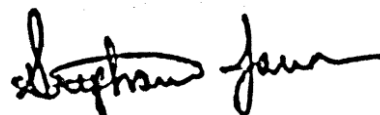
We appreciate HHS's leadership in evaluating the best path to a forward-leaning, industry-supportive approach to regulating healthcare AI. By affirming clinician primacy, adopting scalable risk-based governance, enabling legitimate quality and safety uses of data, and harmonizing expectations across federal programs, HHS can accelerate responsible AI adoption that improves outcomes, reduces clinician and staff burden, and helps constrain systemic costs.

The EHR Association stands ready to collaborate on and support the safe, effective, and equitable deployment of AI in health information technologies and across the healthcare system.

Sincerely,



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Altera Digital Health



Stephanie Jamison
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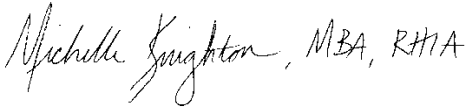
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Established in 2004, the Electronic Health Record (EHR) Association is comprised of 27 companies that supply the vast majority of EHRs to physicians' practices and hospitals across the United States. The EHR Association operates on the premise that the rapid, widespread adoption of EHRs will help improve the quality of patient care as well as the productivity and sustainability of the healthcare system as a key enabler of healthcare transformation. The EHR Association and its members are committed to supporting safe healthcare delivery, fostering continued innovation, and operating with high integrity in the market for our users and their patients and families. The EHR Association is a partner of HIMSS. For more information, visit www.ehra.org.